

# Public Health Insurance in Low- and Middle-Income Countries

*Summary in brief*

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Dupas, P., & Jain, R. (2026). Public health insurance in low- and middle-income countries. In R. Hanna & B. A. Olken (Eds.), *The handbook of social protection: Evidence and new directions for low- and middle-income countries* (Chapter 13). MIT Press.

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## Core question

Publicly funded health insurance has expanded rapidly across LMICs in pursuit of Universal Health Coverage, yet the evidence on its effects is mixed. The chapter asks which design choices and implementation frictions determine whether these schemes actually deliver financial protection and better health.

## How the chapter is organized

- The unmet need for effective health insurance
- Choices in insurance design — financing, eligibility, covered services, provider participation, payment mechanisms
- Provider-incentive challenges — supply-side responses to payment systems and oversight
- Demand-side challenges — enrollment and utilization barriers
- Conclusions and research gaps

## Coverage and take-up

- Coverage averages about 31% across LMICs, but the spread is enormous: roughly 7.9% in low-income countries versus 52.5% in upper-middle-income countries.
- Some national expansions have been dramatic — Rwanda from 40% (2005) to 80% (2019); Indonesia from 41% (2012) to 60% (2017).
- Enrollment is highly price-sensitive: take-up rises substantially when premiums are cut by 30% or more, or eliminated altogether.

## Financial protection and health outcomes

- Effects on out-of-pocket spending are inconsistent. One study documents a 30% decline; most find statistically insignificant effects.
- Three studies find modest reductions in catastrophic expenditure (2–4 percentage points), with some estimates imprecise.
- Health effects are similarly uneven: Gruber et al. (2023) estimate a 2.7% life-expectancy gain from China's rollout, while Mexican and other evaluations find negligible improvement.
- Many programs did not raise healthcare utilization at all, despite nominal coverage.

## Demand-side barriers

- Distance and travel costs, which weigh disproportionately on women's utilization.
- Administrative complexity that deters both enrollment and claim submission.
- Low trust that providers will accept the insurance or that insurers will reimburse.
- Limited awareness of entitlements — even among people already enrolled.

## Supply-side and provider-incentive problems

- Provider moral hazard under fee-for-service: one study finds a 43% cost increase driven by unnecessary prescribing.
- Upcoding and fraud — hospitals reclassify cases to draw higher reimbursement.
- Balance-billing: unauthorized out-of-pocket charges persist where reimbursement rates are set too low.
- Outright refusal of service to insured patients, with refusal rates of 55–69% documented in India.
- Weak oversight capacity, which limits both fraud detection and quality enforcement.

## Conclusions and research gaps

The authors stress that large-scale quantitative evidence on the impact of these design choices in LMIC settings remains severely limited. Their agenda:

- **Supply side** — how providers respond to payment design, select patients, and choose quality levels.
- **Demand side** — the non-financial barriers: awareness, trust, administrative burden.
- **Design evaluation** — how specific policy parameters shape effectiveness and equity.
- **Optimal oversight** — balancing enforcement against administrative cost and unintended consequences.

The overarching claim: financing mechanisms, the definition of covered services, and provider payment structures are not administrative details — they determine whether a scheme improves health and shields households from expenditure risk at all.

*Summary prepared for personal reference. The chapter is open access in the MIT Press edited volume; page numbers were not available at the time of writing.*